

Project Abstract Summary

FAIRBANKS MEDICAL CENTRE LIMITED | CDC-RFA-JG-26-0054 | Your health, our mission.

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Project Title: Last-mile Global Health Security for Uganda: FairBanks will help CHWs, VHTs, clinics, and districts put timely outbreak and priority-disease signals into Ministry of Health systems (including NISS-linked pathways), train frontline workers, and practise 7-1-7 style detect-notify-respond loops so threats are contained closer to source.

Applicant: FAIRBANKS MEDICAL CENTRE LIMITED

Project Director: Racheal Nabukeera, Managing Director and Co-founder

Period: 09/30/2026 to 09/29/2031 | **Funding instrument:** Cooperative agreement | **CFDA:** 93.318

Year 1 federal request (draft, all components): \$10,000,000 | **Component 1 core:** \$2,450,000

Abstract

FAIRBANKS MEDICAL CENTRE LIMITED (FairBanks Medical Centre) is a Uganda-registered health organisation in Kampala. We run a licensed medical centre and FairBanks Community Reach with CHWs and VHTs in Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya and nearby peri-urban communities. Slogan: Your health, our mission. Uganda's 2023 Joint External Evaluation showed progress — and still named hard gaps: community and facility signals that reach decision-makers too late; weak information sharing across sectors; limited surge-ready workforce at subnational level; and incomplete links into national surveillance and emergency structures, including work toward a functional National Integrated Surveillance System (NISS). When those gaps stay open, outbreaks grow before anyone can act. That puts Ugandans at risk first — and Americans at risk later. This project closes the last-mile gap under Ministry of Health leadership. We will not rebuild Uganda's national architecture. We will make community and clinic signals usable inside it. Year 1 focus (Component 1 — Core GHS) has four practical pillars: (1) community and facility surveillance that contributes to MoH/NISS pathways using FairBanks' FCHIP tools; (2) faster community-to-district detection and response using 7-1-7 style timing; (3) CHW/VHT and frontline workforce skills for One Health-aware surveillance and surge readiness; and (4) stronger community-facility links for priority disease programmes (including HIV, TB, malaria, cholera, viral hemorrhagic fevers, mpox, and immunisation) during routine work and outbreaks. Laboratory and border-health work will be done with MoH-aligned partners rather than as a FairBanks-only national lead. Components 2-5 set out contingency surge plans for small and large outbreaks, emerging infectious threats, and humanitarian emergencies. They may be approved but unfunded until CDC releases emergency funds. Expected shared results: shorter detection and response times; more complete community signals in national channels; a better-prepared frontline workforce; and clear handoff of tools and data to government systems. Contact: Racheal Nabukeera, info@fairbanksmedicalcentre.org, +256772849258.